

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

November 23, 2021

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps (b)(6),(b)(7)(C),(b)(7)(F)

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F) MS
Chief of Staff
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CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C (b)(6),(b)(7)(C),(b)(7)(F)
Chief, Investigations Unit
ICE Health Service Corps

FROM: LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

SUBJECT: Mortality Review – Report of Findings
Pablo SANCHEZ-Gotopo, (b)(6),(b)(7)(C)

Executive Summary:

On October 1, 2021, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Pablo SANCHEZ-Gotopo, (b)(6),(b)(7)(C)

(b)(6),(b)(7)(C) During his detention period, ICE housed Mr. SANCHEZ-Gotopo, a 40-year-old Venezuelan male, at Adams County Detention Center (ACDC) in Natchez, Mississippi (MS), from May 21 to October 1, 2021, the date of his death. An autopsy ruled Mr. SANCHEZ-Gotopo's cause of death as pneumonia (an infection of the lungs).

Prior to ICE custody, on an unknown date and location, Mr. SANCHEZ-Gotopo entered the United States without authorization or parole by an immigration officer. On May 17, 2021, U.S.

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Border Patrol encountered Mr. SANCHEZ-Gotopo near Del Rio, Texas, determined him as inadmissible into the United States pursuant to section 212(a)(7)(A)(i)(I) of the Immigration and Nationality Act, and processed him for an Expedited Removal. During Mr. SANCHEZ-Gotopo's initial interview and processing, (b)(6),(b)(7)(C). On May 21, 2021, ERO New Orleans booked Mr. SANCHEZ-Gotopo into ICE custody and transferred him to ACDC for housing, where he remained until his transfer to the hospital on July 28, 2021.

On May 21, 2021, during Mr. SANCHEZ-Gotopo's intake screening, he denied a history of any medical or mental health conditions. A registered nurse (RN) documented negative coronavirus disease 2019 (COVID-19) and tuberculosis (TB) screening results. A medical doctor (MD) ordered Mr. SANCHEZ-Gotopo's rule-out TB chest x-ray (CXR) and rapid COVID-19 nucleic acid amplification test (NAAT); results: negative.

On June 2, 2021, an advanced practice provider (APP) completed Mr. SANCHEZ-Gotopo's initial physical examination and documented normal findings and vital signs (VS), except for an elevated temperature of 101 degrees Fahrenheit [(°F): normal range 95.5–99.9°F]. Mr. SANCHEZ-Gotopo denied any chronic medical conditions but reported a cough and sore throat. The APP ordered a rapid COVID-19 NAAT; results: negative.

Between June 2 and July 28, 2021, ACDC medical staff evaluated and treated Mr. SANCHEZ-Gotopo for cough and sore throat. On July 28, 2021, an MD evaluated Mr. SANCHEZ-Gotopo for shortness of breath and ordered a rapid COVID-19 NAAT; results: negative. The MD referred Mr. SANCHEZ-Gotopo to the Merit Health Natchez (MHN) Medical Center emergency room (ER) via emergency medical services (EMS) for further evaluation.

Between July 28 and September 25, 2021, MHN admitted Mr. SANCHEZ-Gotopo, diagnosed him with (b)(6),(b)(7)(C)

(b)(6),(b)(7)(C) and ordered additional laboratory studies, (b)(6),(b)(7)(C)

(b)(6),(b)(7)(C)

(b)(6),(b)(7)(C). MHN treated Mr. SANCHEZ-Gotopo for numerous disorders; (b)(6),(b)(7)(C) and pneumonia; however, his condition declined throughout his hospitalization.

On September 25, 2021, MHN transferred and admitted Mr. SANCHEZ-Gotopo to Merit Health River Oaks (MHRO) hospital in Flowood, MS, for pulmonary and (b)(6),(b)(7)(C) management. Between September 25 and October 1, 2021, Mr. SANCHEZ-Gotopo's condition continued to deteriorate.

On October 1, 2021, Mr. SANCHEZ-Gotopo experienced agonal breathing (natural gasping reflex when receiving an inadequate supply of oxygen that often indicates approaching death)

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throughout the day. At 5:02 p.m., MHRO medical personnel pronounced Mr. SANCHEZ-Gotopo deceased.

Mortality Finding:

As a result of Mr. SANCHEZ-Gotopo's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined Mr. SANCHEZ-Gotopo's medical care at ACDC was not provided within the safe limits of practice. Specifically, the MRC determined ACDC's care could pose a risk of future adverse outcomes. The MRC's findings included the following: 1) The ACDC medical staff did not perform timely evaluations in response to Mr. SANCHEZ-Gotopo's sick call requests; 2) ACDC's sick call policy permitted the scheduling of evaluations up to two weeks after medical staff's receipt of a sick call request; and 3) On several occasions, the ACDC medical staff did not document the use of an interpreter during Mr. SANCHEZ-Gotopo's evaluations.

The MRC convened on October 28, 2021, and consisted of the following IHSC members: Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) RN, western regional nurse manager, nurse reviewer; Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) MD, clinical director, physician reviewer; Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) family nurse practitioner, APP reviewer; and (b)(6),(b)(7)(C),(b)(7)(F) RN, supervisory compliance health care risk management program manager, risk management reviewer.

The following summary of events was based on: 1) Mr. SANCHEZ-Gotopo's ACDC medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. SANCHEZ-Gotopo's ACDC detention file; 5) Mr. SANCHEZ-Gotopo's autopsy report; 6) a remote site visit and virtual staff interviews conducted by factfinders CAPT (b)(6),(b)(7)(C),(b)(7)(F) and LCDR (b)(6),(b)(7)(C),(b)(7)(F) on October 19 – 21, 2021; and 7) applicable ACDC policies and procedures, and ICE Detention Standards.

Summary of Events

On May 21, 2021, an RN documented Mr. SANCHEZ-Gotopo's primary language as Spanish and completed his intake screening using an interpreter. The RN documented Mr. SANCHEZ-Gotopo denied a history of medical and mental health conditions, normal VS, and a negative TB and COVID-19 screening. Mr. SANCHEZ-Gotopo's accompanying U.S. Customs and Border

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Protection transfer summary did not list any medications or medical conditions. An MD ordered Mr. SANCHEZ-Gotopo's CXR and rapid COVID-19 NAAT; both showed negative results.

On June 2, 2021, an APP completed Mr. SANCHEZ-Gotopo's initial physical examination and documented normal findings and VS, except for an elevated temperature of 101 °F. Mr. SANCHEZ-Gotopo denied any chronic medical conditions but reported a cough and sore throat. The APP ordered acetaminophen and dextromethorphan-guaifenesin (combination medication used to treat cough and congestion).

On June 18 and 23, 2021, Mr. SANCHEZ-Gotopo submitted sick call requests complaining of cough and sore throat. An RN triaged both requests as "routine" (requiring evaluation within two weeks), without performing an evaluation, and referred Mr. SANCHEZ-Gotopo to an APP for further evaluation.

On June 28, 2021, an APP evaluated Mr. SANCHEZ-Gotopo for cough and sore throat, documented lungs clear to auscultation, mouth "moist and pink, oropharynx (back of throat) red, no exudates" and ordered dextromethorphan-guaifenesin and aspirin-chlorpheniramine maleate-phenylephrine bitartrate (combination medication used to treat cough and congestion).

Between June 29 and July 11, 2021, Mr. SANCHEZ-Gotopo did not submit any sick call requests or report any concerns.

On July 12, 2021, an RN evaluated Mr. SANCHEZ-Gotopo for his complaint of a panic attack and coughing. The RN documented a heart rate (HR) of 105 beats per minute [(bpm); normal range 60–100 bpm], otherwise normal VS, including an oxygen saturation level (SpO2) of 100% (normal range 95–100%), clear lung sounds, normal throat appearance, and referred him to an APP. The APP documented a normal physical examination, except for the presence of a dry cough, and ordered acetaminophen-guaifenesin-phenylephrine (combination medication used to treat cough and congestion).

On July 15, 2021, ACDC completed Mr. SANCHEZ-Gotopo's consent form and administered one dose of the Janssen (Johnson & Johnson) COVID-19 vaccine without any complications.

On July 26, 2021, a licensed practical nurse evaluated Mr. SANCHEZ-Gotopo for complaint of a cough and fatigue, documented an HR of 116 bpm, a respiratory rate (RR) of 24 respirations per minute [(rpm); normal range 12–20 rpm], otherwise normal VS, including an SpO2 of 98%, a normal exam, except for "throat is white and patchy," and consulted with an MD. The MD gave a verbal order to perform a CXR and a rapid strep test (used to assist in the diagnosis of a bacterial infection in the throat), both showed negative results, start dextromethorphan-guaifenesin, acetaminophen, and azithromycin (antibiotic), and initiate a routine in-house APP referral for a follow-up evaluation.

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On July 28, 2021, an MD evaluated Mr. SANCHEZ-Gotopo for shortness of breath, documented an SpO2 between 89–92%, a HR of 115, “decreased air movement” in both lung bases, and the presence of oral candidiasis (fungal infection). The MD ordered a rapid COVID-19 NAAT that showed negative results and referred him to the MHN ER via EMS for further evaluation.

On July 29, 2021, an MHN ER MD admitted Mr. SANCHEZ-Gotopo for pneumonia, congestive heart failure [(CHF): condition in which the heart does not pump well and causes fluid to accumulate in the lungs], and hypoxia (inadequate oxygen supply in the tissues to sustain bodily functions). An MD ordered a chest computed tomography scan (diagnostic imaging that provides greater detail than x-rays) that showed “hazy interstitial coarsening throughout both lungs consistent with pulmonary edema” (fluid in the lungs). The MD ordered broad-spectrum antibiotics to treat his pneumonia, an echocardiogram (evaluates the heart’s ability to pump blood) that showed normal results, and initiated a cardiology consult for CHF.

Between July 30 and August 1, 2021, MHN ruled out CHF and a pulmonary embolism (blood clot in the lungs) as Mr. SANCHEZ-Gotopo’s cause of pulmonary edema, and initiated several laboratory tests, including blood cultures (identifies infection in the bloodstream), sputum (identifies infection in the lungs), and blood tests for TB (all normal results).

On August 2, 2021, an MD ordered Mr. SANCHEZ-Gotopo’s COVID-19 reverse transcription-polymerase chain reaction (RT-PCR) test that showed negative results (b)(6),(b)(7)(C)
(b)(6),(b)(7)(C)

On August 3, 2021, an MD diagnosed Mr. SANCHEZ-Gotopo (b)(6),(b)(7)(C)
(b)(6),(b)(7)(C).

On August 5, 2021, Mr. SANCHEZ-Gotopo’s (b)(6),(b)(7)(C)

On August 11, 2021, Mr. SANCHEZ-Gotopo experienced “significant respiratory distress with accessory muscle use” and a right pneumothorax (the presence of air or gas in the cavity between the lungs and chest wall, causing a collapsed lung). An MD intubated (placement of an endotracheal tube into the trachea to support breathing) Mr. SANCHEZ-Gotopo and transferred him to the intensive care unit (ICU) for a higher level of care.

- At approximately 5:01 p.m., an MD inserted Mr. SANCHEZ-Gotopo’s chest tube (device to help reinflate the lung) at the right side of his chest “just superior to the third rib,” with subsequent removal on August 18, 2021.

On August 27, 2021, an MD extubated (removal of the endotracheal tube) Mr. SANCHEZ-Gotopo, placed him on supplemental oxygen therapy via nasal canula (device used to deliver oxygen) titrated to maintain SpO2 above 88%, and documented he remained confused and

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disoriented, despite breathing without ventilatory support. Mr. SANCHEZ-Gotopo's SpO2 ranged between 90-92% with oxygen delivered via nasal canula.

On August 29, 2021, an MD ordered Mr. SANCHEZ-Gotopo's placement on a bi-level positive airway pressure [(BiPAP); method of noninvasive ventilation to support breathing] machine for hypoxia. MHN alternated between a BiPAP and Vapotherm (mask-free respiratory device to support breathing) as needed to maintain adequate oxygenation.

On September 5, 2021, an MD consulted with Mr. SANCHEZ-Gotopo's family members and changed his full code status to do not resuscitate (legal order indicating a person should not receive cardiopulmonary resuscitation if that person's heart stops beating) and do not intubate.

On September 15, 2021, Mr. SANCHEZ-Gotopo's condition declined, and his repeat COVID-19 RT-PCR test showed positive results.

On September 18, 2021, Mr. SANCHEZ-Gotopo's family requested the following: "no new interventions, medication changes, or increase in oxygen level; no routine labs; continue current medical management only."

On September 25, 2021, MHN transferred and admitted Mr. SANCHEZ-Gotopo to MHRO for pulmonary and (b)(6),(b)(7)(C)

On September 28, 2021, Mr. SANCHEZ-Gotopo's condition deteriorated, and he experienced decreased kidney function, hypotension (low blood pressure), and tachycardia (elevated HR).

On September 29, 2021, after further discussion with the MHRO MD, Mr. SANCHEZ-Gotopo's family requested to only continue with "comfort measures." As a result, the MD discontinued all medications, future laboratory studies, and ordered morphine (pain medication) and lorazepam (anti-anxiety medication) to aid in comfort.

On October 1, 2021, Mr. SANCHEZ-Gotopo experienced agonal breathing throughout the day.

- At 7:59 a.m., an RN documented the following VS: HR of 100 bpm; RR of 23 rpm; blood pressure (BP) of 73/26 millimeters of mercury [(mmHg); normal range 90/60 – 120/80 mmHg]; SpO2 of 99% on 15 L/min via non-rebreather (NRB) mask.
- At 12:15 p.m., an RN documented the following VS: HR of 101 bpm; RR of 20 rpm; BP of 67/21 mmHg; SpO2 of 99% on 15 L/min via NRB mask.
- At 4:10 p.m., an RN documented the following VS: HR of 83 bpm; RR of 19 rpm; BP of 55/18 mmHg; SpO2 of 100% on 15 L/min via NRB mask.
- At 5:02 p.m., MHRO medical personnel pronounced Mr. SANCHEZ-Gotopo deceased.

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The final autopsy report revealed Mr. SANCHEZ-Gotopo's cause of death was pneumonia and (b)(6),(b)(7)(C) COVID-19 infection as "other significant conditions." The coroner determined Mr. SANCHEZ-Gotopo's manner of death was natural.

Detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and the National Commission on Correctional Health Care (NCCHC) Standards, 2018.

The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) ACDC's nursing assessment protocols and procedures provide comprehensive guidance to nursing staff; and 2) An APP reviews all intake screenings to identify detainees who require an expedited health appraisal not previously referred by nursing staff.

Weaknesses

1. Patient Care

- On June 18 and 23, 2021, the medical record did not include any evidence to confirm Mr. SANCHEZ-Gotopo received a face-to-face encounter within 24 hours of each of his sick call requests.
- Between June 2 and July 12, 2021, ACDC medical staff evaluated Mr. SANCHEZ-Gotopo for complaints of cough and sore throat on three occasions, and treated him for cold symptoms. Despite his abnormal VS, ACDC medical staff did not complete further evaluations to rule out other causes of his symptoms.
- An APP evaluated Mr. SANCHEZ-Gotopo 11 days after he submitted his initial sick call request on June 18, 2021. ACDC's policies and procedures allow medical staff to schedule appointments up to two weeks after triaging the sick call request.
- On July 26, 2021, an LPN evaluated Mr. SANCHEZ-Gotopo, reported assessment to an MD and received verbal orders; however, the MD did not perform an evaluation.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section S. *Sick Call*.
- NCCHC 2018 standards: J-E-07, *Nonemergency Health Care Requests and Services*.
- NCCHC 2018 standards: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.
- ACDC Policy: 13-71, *Physician Orders and Patient Care Protocols*.

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- ACDC Policy: 13-71BB, *Nursing Referral Guidelines*.

2. Communication

- On multiple occasions, medical staff did not document the use of an interpreter when evaluating Mr. SANCHEZ-Gotopo.
- On May 24 and June 2, 2021, ACDC medical staff provided Mr. SANCHEZ-Gotopo with written information to reflect “test results are essentially within normal limits” but did not write the names of the tests in plain English or terms commonly understood by laypersons.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section E. *Translation and Language Access for Detainees with Limited English Proficiency*.

3. Documentation

- There is no documentary evidence the clinical director reviewed Mr. SANCHEZ-Gotopo’s comprehensive health assessment.
- On multiple occasions, the date and time of the clinical encounter was not documented.
- Several clinical encounters contain signatures dated days or weeks after the encounter occurred. Mr. SANCHEZ-Gotopo’s printed medical record does not list the actual date these specific clinical encounters occurred.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section BB. *Medical Records*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section M. *Comprehensive Health Assessment*.
- NCCHC 2018 standards: J-A-08, *Health Records*.
- NCCHC 2018 standards: J-E-07, *Nonemergency Health Care Requests and Services*.

4. Policy

- ACDC’s sick call policy did not contain site specific information and contained conflicting information regarding sick call frequency and evaluation timeframe. It is unclear whether the policy requires nursing staff to collect sick call requests five or seven days per week, and whether medical staff complete an evaluation within 24 or 72 hours of sick call request receipt.

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Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section S. *Sick Call*.
- NCCHC 2018 standards: J-A-05, *Policies and Procedures*.
- NCCHC 2018 standards: J-E-07, *Nonemergency Health Care Requests and Services*.
- ACDC Policy: 13-80, *Sick Call*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, ACDC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.